



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : هاله حسن العزب صالح
المحافظة : الغربية
الحى / المدينة : السنطة
العنوان : -
رقم البطاقة : 27009231601723 رقم التليفون : 01028764724
العمر (تاريخ الميلاد) : 23-09-1970 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 22-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الاورام

خطة العلاج : علاج مبادرة سرطان الثدي مثبط CDK4/6 و فلفسترات
[14469]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هاله حسن العزب صالح National ID: 27009231601723	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الى CDK4/6 inhibitor and fulvestrant

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

BQHEYA

Radiology Department

بنية

PATIENT NAME: HALA HASSAN ELAZAB SALEH

GENDER: Female

DATE : 12-Jan-2026 03:15 PM

PATIENT ID: 2021160162
AGE: 55 y
ORDER:
3723566,3723567,3724112

FULL FIELD DIGITAL MAMMOGRAPHY

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Targeted ultrasound

FINDINGS:

There are scattered areas of fibroglandular density (ACR b).

LEFT BREAST:

- Rather stationary UOQ spiculated dense mass, measuring 6.5x6 cm in diameter 15 mm from skin and 5.8cm from the nipple, targeted ultrasound revealed corresponding 2:00 spiculated mass with internal vascularity on color Doppler.
- LOQ small irregular nodule is seen more appreciated in the current study, being 6.5cm inferior to the aforementioned mass and 7cm from nipple, targeted ultrasound revealed corresponding small irregular hypoechoic nodule surrounded with hyperechoic desmoplastic reaction, located at 3:00, measuring about 0.8x0.6cm in diameter.
- Mild areolar and paraareolar skin thickening are still noted with subtle nipple retraction
- No pathologically enlarged axillary lymph nodes.

RIGHT BREAST:

- No spiculated mass lesions or grouped microcalcifications.
- Targeted Ultrasound of outer quadrants revealed no suspicious lesions.
- Scattered round calcifications.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Rather stationary left breast UOQ pathologically proved malignant mass (BIRADS 6) along with LOQ newly appreciated small suspicious nodule.
- Right breast benign findings (BIRADS 2).

PATIENT: HALA HASSAN ELAZAB SALEH, 2021160162, Female, 55 y

Report version 1, 12-Jan-2026 05:19 PM

MRN:	202160162	Patient Name:	HALA HASSAN ELAZAB SALEH
D.O.B:	23/09/1970	IC No.:	27009231601723
Sex:	Female	Location:	External Clinic

SURGICAL PATHOLOGY
Biopsy Small - (1 Site)

Specimen Id	2107120243	Collection Date	12/07/2021 01:05 PM
Accession No	SUR21-5266	Received Date	12/07/2021 01:06 PM
Sample Type	Tissue	Doctor	Abd ALLAH Mohamed Gamil

Clinical Information

Lt. Breast, UOQ mass.
Gross Picture
5 cores, 0.5-1.5 cm
Microscopic Picture
A case of invasive duct carcinoma, grade II. Stroma is desmoplastic with mild inflammatory cell reaction. There is focal DCIS component, I

Diagnostic Summary

Tumour type:
IDC, grade II
Laterality
Left

Specimen type:

Tru-cut biopsy
Location
Upper outer quadrant

Mitosis score (1-5):

2
Tubular formation score (1-5):

Nuclear grade (1-3):

2
Histological grade (1-3):

2
Peritumoural lymphovascular invasion

Diagnosis

Lt. breast, tru-cut biopsy:
Invasive duct carcinoma, grade II.

Comments

ER, PR and Her 2 are ordered.
Biopsy reporting category

B5b= Invasive cancer

Signature

MONA MOHAMED MAMDOH

Co-signed by:

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Name

54

ID Number

ID Type

Gender

Nationality

Age 55Year(s)

Visit Date

Diagnosis

Postmenopausal 54 vs with MBC (bone) T4 N2 M1

received TAM/ zoladex from 8/2021 10/2022--> PD--> AC x6--> AI /Zometa 2/2023

CT CAP 3/2024: stationary course since the last study dated 16/10/2023.

CT CAP: 15/5/2025

stationary course with no newly developed lesions.

The examined skeleton shows stationary course as regard:

- Widespread sclerotic osseous lesions.
- Collapsed D3 vertebra seen indenting the cord. (previously irradiated)
- Vertical cleavage of D10 vertebral body.

Bone scan:

partial regression of metastatic deposits

7-2-87
Catalina
Catalina Clinics

Written By

Approved By

Medical Director

AMIRA Diyaa Abd-Elmoniem DARWISH

[Handwritten signature]
Medical Department

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناتيونال - مصر -معمل الباثولوجي

Patient Name	ملاء حسن العرب	Case No.	357643
Sex/Age	Female 55 Y	Patient No.	25798
Registered Date	12/8/2026 11:34:17 AM	Referred By Dr.	Out Patient
Validation Date	25/8/2026 01:45:54 PM	Branch	Gustave Roussy
Reporting Date	14/9/2026 10:59:35 AM	Pathology No.	SR-930/26

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Block revision.

Gross Examination:

- One paraffin block labeled by (4828/23) was received.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 1).
- DCIS: Absent.
- Micrometastasis: Present.
- Angiolymphatic invasion: Absent.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (score 0+).
- Ki67%: 15%.

Conclusion:

Left breast cancer, Block revision:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Ultra low (score 0+).
- Ki67%: 15%.

شقة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Al-Misr El Qadeema - Cairo, Egypt



Gustave Roussy International-Egypt- Pathology Lab
جوستاف رويسى انترناشيونال - مصر -معمل الباثولوجي



Patient Name	مالة حسن الزرب	Case No.	357643
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****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 10%; 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Choneim, MD.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mar El Qadima - Cairo, Egypt

09/22/2026

MDT FORM

Name:	Male Sex	MRN:	13137
Age:	56 Y	ID:	27009231601723
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 6/8 **HER2:** 0 **KI67:** 15

Sonomammography:

Mets Workup:

History:

55 years old female patient, pre menopausal received tam + zoladex from 8/2021 to 11/2022 >> PD. received AC*6 with regression then kept on maintenance AI + zometa from 2/2023 to 1/2026.

ct cap 1/2026 + bone scan >> SD but clinically progressed with severe back pain

MRI cervical >> multiple vertebral osseous lesion likely mets with c5+d3 cord compression
recommendation for faslodex + cdk4/6 inhibitor but not received
received palliative RTH to C4>D3

Recommendations:

shifted to faslodex + ibrance
ct cap + MRI whole spine + bone scan as base line in our center

MDT Outcome & Plan

shifted to faslodex + ibrance
ct cap + MRI whole spine + bone scan as base line in our center

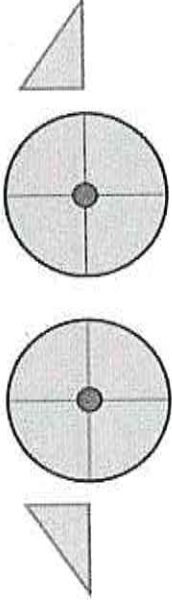
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

د. محمد كركاشي

R

L



TNM

